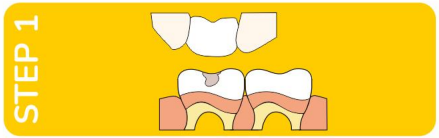
BIOFLX CROWNS - INSTRUCTIONS FOR USE

INTENDED USE and INDICATIONS: Bioflx Crowns are designed for full coverage restoration of damage or decayed teeth

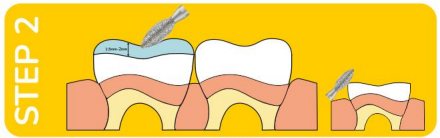
posterior technique

OCCLUSAL EVALUATION & CROWN SELECTION



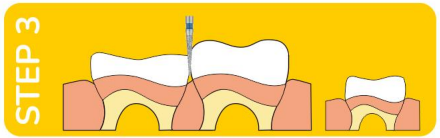
The occlusal relationship should be evaluated before the application of the rubber dam. Approximate the mesio distal dimension to determine the space available between the adjacent teeth, then choose the smallest Bioflx crown that will restore the proximal contacts.

OCCLUSAL REDUCTION



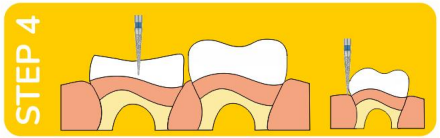
Local anestheia is usually necessary. Occlusal reduction should be uniform 1.5-2.0mm following the natural occlusal contours. Reduce buccal and lingual cusps, including the central groove and marginal ridges using flame shape bur

PROXIMAL REDUCTION



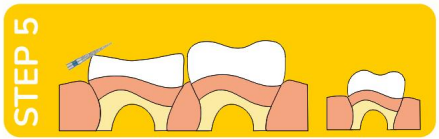
Using a tapered fissure bur slice through the proximal contact of mesial and distal surface without creating a ledge in the proximal surface or damaging the adjacent tooth. Proximal slices should slightly converge toward the occlusal surface.

BUCCAL OR LINGUAL REDUCTION



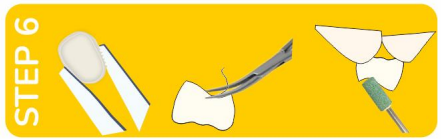
Remove buccal bulges and lingual convexities while maintaining a snug fit.

ROUND OF LINE ANGLES



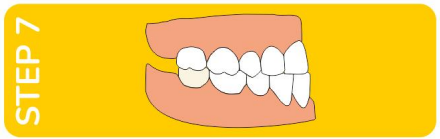
Round all sharp edges and line angles. Make a final check of the preparation.

ADJUSTING BIOFLX



Crimping the crown is **NOT** recommended. Slight contouring may be done with Howe Pliers. If the crown is too small, use the next larger size crown or further reduce the tooth circumferentially. If the crown is in hyperocclussion, the tooth preparation may have a ledge, may need more occlusal reduction; or the proximal contact may be too tight with the adjacent tooth. Excessive gingival blanching means that the crown is too long or too bulky. A properly fit crown extends approximately 1.0mm into the sulcus and may be trimmed with C&B scissors or a stone as needed. No other areas of the BioFlx crown should be adjusted or crimped.

TRIAL FITTING



Try on posterior crowns by placing from lingual to buccal. The crown should not actively bounce back after fully seating, and the margin should be placed 0.5-1.0mm subgingivally. A snug/active fit should provide some resistance to dislodgment of the crown. Check occlusion, occlusion should not be high. **THE BIOFLX CROWN SHOULD NEVER BE THE FIRST POINT OF CONTACT IN OCCLUSION.**

⚠ Do not use crown remover, use spoon excavator to remove the tried crown gently, make sure the crown isn't damaged in the process of removal.

CEMENTATION



The prepared teeth and the crown should be cleaned of any saliva, blood or debris, and any gingival hemorrhage reasonably controlled prior to cementation Self-setting RMGI (FujiCEM® 21) or GI (Fuji I®1, Ketac™2) may be used to cement the BioFlx crown. Fill the crown approximately two-thirds full with cement so that excess cement will flow out from the margins during cementation minimizing any voids. Run a knotted floss through the contacts to remove any residual interproximal cement. Check occlusion.

CAUTION: **CROWNS CEMENTED FOR INTRAORAL SERVICE should NOT be cleaned and sterilized to use for another patient**

PRECAUTIONS:

- Do not Keep the Occlusion High
- Crowns that are reprocessed after cementation in another patient may cause infection resulting in possible long term health effects.
- Dental crowns may cause artifacts in MRIs, MRI technician should be notified of any dental crowns.

STORAGE:

Crowns should be stored in a clean storage container.

Kids-e-Dental
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MD Medical Device | Non-Sterile | Caution | Rx only Prescription Only | Do Not Reuse